M26 Metoclopramide

Scope of Practice	P
Classification	• Antiemetic
Usual Presentation	• 10mg/2ml ampule
Mechanism of Action	• Antagonizes dopamine-2 receptors in the chemoreceptor trigger zone (CTZ) reducing nausea. • Metoclopramide also (weakly) blocks stimulation of the 5-HT3 receptors in the gastro-intestinal system further reducing nausea and increasing gastric motility.
Indications	• Nausea and Vomiting
Contraindications	• Known allergy • Epilepsy • Parkinsons Disease • Tardive dyskinesia • Suspected gastro-intestinal hemorrhage or obstruction • Pheochromocytoma • Recent gastro-intestinal surgery • Age <1 year • Allergy to procainamide
Precautions	• Pediatrics (extrapyramidal effects)
Side Effects	• Extrapyramidal effects (common in children) • Rash • Hypotension • Dyspnea • Drowsiness and dizziness
Adult Dose & Administration	• 10mg IV/IM (slow IV over 3 minutes, or diluted in 100ml normal saline and administered as an infusion)
Pediatric Dose & Administration	• Generally not recommended for routine pediatric administration (significant risk of extrapyramidal effects) • 0.1mg/kg IV/IM (slow IV over 3 minutes)
Use in Pregnancy and Lactation	• Pregnancy: Safety has not been established. Routine administration in pregnancy is not recommended. • Lactation: Safety has not been established. Routine administration in lactation is not recommended. If essential to be administered, the mother should be advised to stop breast feeding until advised by their physician.
Additional Information	• Metoclopramide should not be mixed with morphine. • Metoclopramide should not be administered prophylactically after opiate administration. Only administer if nausea occurs.